SYNTHETIC RECORD - FOR DEMONSTRATION ONLY
Patient: Dana Whitfield. DOB: 1979-03-22. MRN: SYN-100481.
Visit date: 2026-09-10. Clinic: Lakeside Family Medicine (fictional).
Chief complaint: low back pain radiating down the left leg.
Symptoms began on 2026-07-20 after lifting boxes; pain has persisted for 7 weeks.
Pain is rated 7/10 and worsens with sitting and bending.
Patient reports numbness along the lateral left foot.
Physical therapy twice weekly from 2026-07-27 to 2026-09-07 (6 weeks), with a home exercise program.
Naproxen 500 mg twice daily since 2026-07-22 and activity modification at work.
PT discharge note states no meaningful improvement in radicular pain.
No fever, weight loss, bowel or bladder changes, or history of cancer.
Exam: lumbar paraspinal tenderness, flexion limited to 40 degrees.
Straight leg raise positive on the left at 35 degrees.
Decreased sensation to light touch in the left S1 dermatome.
Left ankle reflex 1+, right 2+.
Strength 5/5 in both lower extremities except left plantarflexion 4/5.
Gait antalgic; heel and toe walking difficult on the left.
Assessment: left S1 radiculopathy, likely L5-S1 disc herniation.
Persistent symptoms despite 6 weeks of conservative management.
Plan: MRI lumbar spine without contrast (CPT 72148).
MRI findings will determine referral for epidural steroid injection versus spine surgery consultation.
Continue home exercise program and naproxen.
Return precautions reviewed for new weakness or bladder symptoms.
Follow-up in 2 weeks or after imaging.
Electronically signed: Morgan Ellery, MD (fictional).